

Pediatric

P-04 Pediatric Bradycardia

Signs and symptoms of shock

ALOC, hypotension (age appropriate), capillary refill >2 seconds,
diaphoresis, SOB

Stabilize airway using appropriate BLS adjunct as necessary

Oxygen – high flow via NRB

Assist ventilations with BVM as necessary

Pulse Oximetry

Cardiac Monitor

IV/IO access

Consider **NS** 20mL/kg bolus for hypotension

Identify and treat reversible causes

- Hypovolemia
- Hypoxia
- Hydrogen ion acidosis
- Hypo/hyperkalemia
- Toxins
- Tamponade, cardiac
- Tension pneumothorax
- Trombosis
- Trauma

Persistent bradycardia with
hypotension and shock?

No

Supportive care
Reassess frequently

Yes

For HR <60
Chest compressions
Minimize interruptions and change
personnel often to avoid fatigue

Epinephrine 1:10,000 0.01mg/kg (0.1mL/kg) IV/IO
q3-5min

For suspected increased vagal event or primary AV block

Atropine 0.02mg/kg IV/IO
May repeat once
Minimum single dose 0.1mg
Maximum single dose 0.5mg

Consider transcutaneous pacing
Consult Base Hospital Physician

DISRUPTED COMMUNICATIONS

In the event of a “disrupted communications” situation, Solano County Paramedics may utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.